Maltreated children in out-of-home care: The relation between attachment quality and internalizing symptoms

Ashley A Chesmore,[1] Lindsey M. Weiler,[1] Lisa J. Trump,[1] Ashley A. Landers,[1] and Heather N. Taussig[2]
[1] Family Social Science, University of Minnesota Twin Cities; [2] Department of Pediatrics, University of Colorado

Background

Maltreated Children in Foster Care

- Exposure to maltreatment is a significant risk factor for the development of internalizing symptomology (Pears, Kim, & Fisher, 2008). Children in foster care may be at an even greater risk due to their cumulative exposure to additional adverse events (e.g., caregiver disruptions; Raviv, Taussig, Culhane, & Garrido, 2010).
- Because not all foster children develop internalizing disturbance, it is imperative to identify factors associated with variations in children's internalizing symptoms.

Attachment Quality with Caregivers

- Positive parent-child relationships often serve as protective factors; however, maltreated children in foster care commonly experience insecure attachments with biological (bio) parents (Baer & Martinez, 2006).
- If maltreated children are able to develop quality attachments with foster parents, this relationship may buffer the negative impact of poor attachments with bio parents on children's internalizing symptoms.

Gaps in Current Literature

- Few studies have examined the association between attachment quality and child internalizing symptoms among children in foster care.
- Unclear if children's attachment quality with foster parents is related to children's internalizing symptoms
- Few studies have accounted for placement experience factors (e.g., placement type, caregiver transitions)

Research Questions

1. What is the relationship between child-reported attachment quality (with the bio parent and foster parent) and child internalizing symptoms?
2. Does attachment to foster parents moderate the relationship between children's attachment to bio parents and children's internalizing symptoms?

Sample

Participants are from the Fostering Healthy Futures (FHF) Study (PI: Dr. Heather Taussig). FHF is an ongoing and comprehensive investigation of the effects of a mentoring and skills-based preventive intervention for maltreated preadolescents recently placed in foster care.

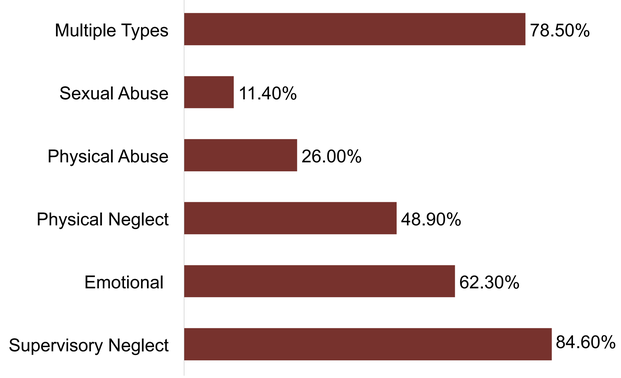
Age and Sex

- 9 -11 years old; 51.1% male

Race and Ethnicity

- 47.5% Hispanic, 47.1% European American, 26.8% African American, 13.4% Native American, 2.6% Asian American, 1.6% Pacific Islander (non-exclusive categories)

Figure 1. Forms of Maltreatment



Measures

Demographic and maltreatment variables

1. Child age and sex (1 = male; 0 = female)
2. Types of maltreatment coded as present or absent using the Maltreatment Classification System (Barnett, Manly, & Cicchetti, 1993).

Placement experience variables

1. Placement type (1 = foster care; 0 = kinship care)
2. Number of caregiver transitions
   (*R* = 1.0 – 11.0; *M* = 2.7; *SD* = 2.0)
3. Frequency of contact with bio mother
   (*R* = 0.0 – 3.0; *M* = 2.15; *SD* = 1.21)
4. Length of time in foster care (in months)
   (*R* = 1.0 – 15.0; *M* = 7.10; *SD* = 3.43)
5. Length of time in current placement (in months)
   (*R* = 1.0 – 24.0; *M* = 6.09; *SD* = 3.80)

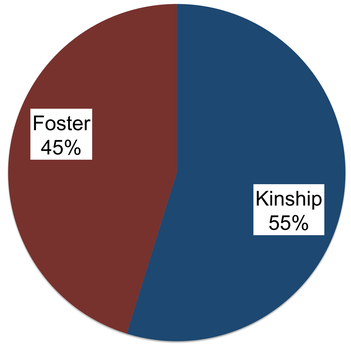
Attachment quality. Assessed using the Inventory of Parent and Peer Attachment – Short Form (Gifford-Smith, 2000). Sample items include questions asking children to rate whether their caregivers, "help me with my problems" And "care about me."

- Child-report of AQ with bio parent
  (*R* = 15.0 – 45.0; *M* = 38.6; *SD* = 6.8)
- Child-report of AQ with foster parent
  (*R* = 16.0 – 45.0; *M* = 38.6; *SD* = 5.0)

Internalizing symptoms.

1. Child-report of anxiety was assessed with the Revised Children's Manifest Anxiety Scale (Reynolds & Richmond, 2000)
   (*R* = 25.0 – 79.0; *M* = 51.4; *SD* = 10.6)
2. Child-report of depression was assessed with the Trauma Symptom Checklist for Children (Briere, 1996).
   (*R* = 32.0 – 84.0; *M* = 46.5; *SD* = 9.9).
3. Foster parent report of internalizing problems was assessed with the Child Behavior Checklist (Achenbach & Rescorla, 2001)
   (*R* = 33.0 – 86.0; *M* = 59.4; *SD* = 11.6)

Figure 2. Placement Type



Results

Table 1 *Hierarchical regression models predicting child internalizing symptoms*

	Anxiety			Depression			Internalizing		
	β	R^2	$R^2\Delta$	*β*	R^2	$R^2\Delta$	*β*	R^2	$R^2\Delta$
Step 1:		.021			.017			-.004	
Age	-.15**			-.14**			.01		
Sex	-.09†			-.07			.02		
Step 2:		.021	.024		.034	.040†		.027	.056*
Placement type (1 = foster care)	.06			.06			.18**		
Step 3:		.132	.112***		.136	.103***		.057	.034**
AQ with foster parent	**-.29*** **			**-.28*** **			**-.14** **		
AQ with bio parent	**-.15** **			**-.14** **			**-.12***		
Step 4:		.132	.002		.140	.006†		.055	.001
Foster AQ X bio AQ	.05			.08†			-.03		

AQ = Attachment quality
† *p*<0.10, **p*<0.05, ** *p*<0.01, *** *p*<0.001

Analysis Plan

Hierarchical regression models were used to specify relationship between parent-child attachment and child internalizing symptoms

Variables entered in 4 steps
Step 1: Child sex and age
Step 2: Maltreatment and placement variables
Step 3: Parent-child attachment variables
Step 4: An interaction variable, child attachment to bio parents X child attachment to foster parents

Continuous variables were centered at the mean

Note: **Only variables that were marginally or statistically significant are presented.**

Discussion

Main Findings

Children who reported greater attachment quality with caregivers tended to exhibit fewer internalizing symptoms.

Children's attachment quality with foster parents was more strongly associated with children's internalizing symptoms than children's attachment with bio parents.

While both relationships are important to attend to, children's relationships with the caregiver with whom they are currently residing with may be especially important for maltreated children's current internalizing symptomology.

Study Limitations

- Cross-sectional
- Results are not generalizable to maltreated children who have been placed in foster care for a longer period of time
- Attachment quality assessed through child-report only (Potential shared method variance)

Conclusions

- If our findings are replicated in a longitudinal design, it may suggest that clinical interventions should focus on promoting positive attachment experiences between maltreated children and both their bio parents and foster parents.
- It would be valuable to assess the association between children's attachment and internalizing symptoms over time in order to determine if one relationship (i.e., bio parents or foster parents) is more predictive of children's current and later mental health functioning.
- If future studies find that a positive relationship with a foster parent ameliorates the negative impact of poor early parental care, it may suggest that a positive relationship with a foster parent serves as a protective factor for maltreated children recently removed from their homes.
- Intervention programs that attempt to form attachment relationships between foster parents and children may be one avenue to prevent increased internalizing problems.

